

Q Why would my baby need medical care?

There are many reasons why a newborn might need medical care. Some relate to premature births and others may occur in full-term babies. The following is a list of the most common reasons:

» **Respiratory disorders:** your baby may need extra oxygen or help with breathing in the short term if his respiratory system hasn't fully formed yet, or because he has an infection or meconium in his lungs.

» **Temperature control:** even full-term babies are very bad at regulating their temperatures after birth. Your baby may need to go into an incubator until his body systems are better developed.

» **Hypoglycemia:** this describes a condition when your baby's glucose levels



fall too low. This can be the case in babies of mothers with diabetes, or in premature babies. Your baby's brain and body need glucose to function, so if he's unable to raise his sugar levels through feeding, he may have to go onto a nasogastric feeding tube to correct the imbalance.

» **Jaundice (see p.322):** this is a relatively common newborn condition in which a baby's liver doesn't break down bilirubin (a by-product of red blood cells) in his blood. It occurs in around 60 percent of full-term babies, and approximately 80 percent of premature babies. Most cases are harmless. Your baby may need

Your reassuring presence Staying close by, talking to, and stroking your baby will help him to feel calm, and is a comfort for you, too.

treatment if he's premature, or has particularly high levels of bilirubin in his blood. Exposure to light or phototherapy might be recommended.

» **Patent ductus arteriosus:** when your baby is born, the vessel that keeps blood-flow away from the lungs during pregnancy should close off. If this doesn't happen—your baby's lungs and heart can be strained (patent ductus arteriosus). See p.323.

Q When is a baby considered premature and why do some babies arrive early?

A **premature (or "preterm") baby** is a baby born before 37 weeks of pregnancy. In the US, approximately 11 percent of births (almost eight percent in the UK) are premature. It's unusual for there to be a single, clear reason why a baby arrives early; usually preterm labor is the result of a combination of factors. For up to 30 percent of premature births, the cause doesn't ever become clear. In another 25 percent of premature births, labor is medically induced before the baby reaches full term because a problem has been detected. This means that an early delivery is the safest option for the mother and/or baby. The following list sets out some of the reasons why babies are born prematurely:

» **If you have a pregnancy condition** such as preeclampsia (see p.144), where your baby's growth may be restricted, or gestational diabetes (see p.145), where your baby can grow very large, you may be induced early on.

» **Placental problems**, including placenta previa (see p.147) and placental bleeding, can mean that labor needs to be induced early.

» **You may have a weak cervix**, which cannot support the baby and membranes to full term.

» **If amniotic fluid begins to leak** from the sac around the baby, known as premature prolonged rupture of membranes, your baby is at risk of infection, and labor may need to be induced.

» **Having had a premature baby** in a previous pregnancy or having had a previous late miscarriage increases your risk of premature labor.

» **Carrying twins or more** adds to pressure on the uterine muscles. Similarly, excess amniotic fluid can distend the uterus and trigger labor.

» **Preexisting maternal conditions**, such as high blood pressure, diabetes, or kidney disease, and irregularities in the shape or structure of the uterus, can mean labor is induced earlier.

» **Being younger than 25 or older than 35** increases the risk of premature labor.

» **Lifestyle factors**, such as smoking, drinking alcohol, taking drugs, or poor nutrition in pregnancy make premature labor more likely.

Q What will my premature baby look like?

This depends upon how close to your due date you were when you went into labor. A baby born at 36 weeks will look as you might expect a newborn to look, except smaller. A more premature baby might have very pink or reddish skin, because the fat layer under the skin hasn't formed, and he may be covered in lanugo, the soft, downy hair that protects the skin in the uterus and is shed toward the end of pregnancy. His head may seem large for his body, and, if very premature, his eyelids may be fused. The top of his skull will be soft (and his fontanelle large), since the flat bones haven't fully formed over the sides and top of his head. His limbs may seem fragile, and since his muscle tone is still quite weak, he may not be curled up as term newborns often are since he can't hold his knees to his chest. His facial features will be fully formed and he may have hair on his head.